AC-08 Adult Polymorphic Wide-Complex Tachycardia

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for adults age 16 years or older with a pulse and polymorphic wide-complex tachycardia. Sustained polymorphic VT is unstable until proven otherwise.

Polymorphic VT and torsades pathway

1. Confirm rhythm + pads	Beat-to-beat QRS variation; support airway/ventilation, oxygen if hypoxemic, apply pads immediately, and do not delay shock.
2. Sustained polymorphic VT	Treat as unstable regardless of apparent perfusion.
3. Immediate shock	Unsynchronized 200 J biphasic; verify the manufacturer setting, escalate according to device, and never attempt synchronization.
4. Pulse lost	Begin CPR immediately and open AC-09 VF/Pulseless VT.
5. Rhythm terminates	Obtain strip/12-lead and determine whether baseline QT is prolonged.
6. Long-QT/torsades recurrence	Magnesium sulfate 1-2 g IV over 15 minutes; may repeat once.
7. Normal-QT recurrence	Treat suspected myocardial ischemia; lidocaine 100 mg IV/IO initial dose.

Key treatment and handoff

Item	Approved action
After conversion	Pads remain attached; continuous monitoring, frequent vital signs, early notification, and anticipation of recurrence or VF.
Underlying cause	Review drugs, electrolytes, ischemia, bradycardia, and QT prolongation without delaying electrical treatment.

For sustained polymorphic VT, do not delay unsynchronized defibrillation to attempt a 12-lead ECG, medication, or synchronization.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Do not give adenosine, diltiazem, verapamil, metoprolol, digoxin, or another AV-nodal blocker for polymorphic wide-complex tachycardia.

- Do not use amiodarone for torsades or clearly prolonged-QT polymorphic VT.

Linked pathways

Pathway	Use when
AC-04	Suspected ischemia/ACS
AC-09	VF/pulseless VT if pulse lost
AC-10	Post-resuscitation care after ROSC

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.